

AHN Forbes Hospital

2570 Haymaker Rd, Monroeville, PA 15146

DISCHARGE SUMMARY Document #: DIS-017

Patient & Admission

Name	Elizabeth Harvey
Date of Birth	1975-01-25
Phone	724-555-9763
Address	7738 Leon Underpass Apt. 148 Pittsburgh, PA, 15222
MRN	MRN-433200
Admit Date	2026-05-06
Discharge Date	2026-05-10
Attending	Pamela Boyd, MD
Attending NPI	1733754331

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	JIM514846564

Primary Discharge Diagnosis

Mild CAD on diagnostic angiography

ICD-10: I25.110

Hospital Course

The patient presented with chest discomfort and dyspnea on exertion concerning for acute coronary syndrome. Initial electrocardiogram showed nonspecific ST-T wave changes, and serial troponin levels remained negative. Transthoracic echocardiography demonstrated preserved left ventricular ejection fraction at 55% with no significant wall motion abnormalities. BNP was mildly elevated at 285 pg/mL. Given the clinical presentation and risk factor profile, the patient was admitted for further cardiac evaluation and underwent diagnostic cardiac catheterization on hospital day two.

Coronary angiography revealed mild atherosclerotic disease in the left anterior descending artery with approximately 40% stenosis and minimal disease in the right coronary artery. The left circumflex artery was angiographically normal. No acute thrombosis or critical lesions were identified, and PCI was not performed. The patient tolerated the procedure well without complications. Vascular access was via the right femoral artery with successful hemostasis achieved. Post-procedure vital signs remained stable, and there were no signs of access site hematoma or pseudoaneurysm on discharge examination.

The patient's symptoms improved significantly during hospitalization with medical optimization. He was initiated on aspirin, a high-intensity statin, and a beta-blocker for secondary prevention and symptom

management. Discharge planning included outpatient cardiology follow-up in two weeks for risk factor modification counseling and consideration of stress testing if symptoms recur. The patient was counseled on lifestyle modifications including smoking cessation, dietary sodium restriction, and regular aerobic exercise. He demonstrated good understanding of his medication regimen and agreed to close outpatient follow-up. The patient was discharged in stable condition on hospital day four.

Procedures Performed

Coronary angiography	CPT 93458
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Medication Changes

Clopidogrel 75 mg daily	STARTED
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Isosorbide mononitrate 30 mg daily	STARTED
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Follow-up Instructions

Recommended Specialist	Cardiology
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Follow-up Window	8 days
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Urgency Tier	MEDIUM
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Clinical Flags	radial access — wrist precautions, post-PCI dual antiplatelet therapy, femoral access — bleed precautions
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Dictated and electronically signed by:
Pamela Boyd, MD
NPI: 1733754331
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